



Occupational Therapy Rehabilitation Ward Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Kalamunda Group	Rehabilitation Ward	Occupational Therapy

2. Purpose

This policy outlines the framework for Occupational Therapy (OT) services on the Rehabilitation Ward, ensuring consistent and comprehensive OT assessments, interventions, and discharge processes to support functional recovery for patients and safe reintegration into the community.

3. Procedure

Administrative

- Carry designated pager during shift.
- Complete clinical documentation in line with AKG Clinical Documentation Policy and requirements in AKG OT procedures.
- Ensuring all Allied Health Statistics are completed by the end of the shift
- Ensure all relevant handovers are completed for the next rostered OT (including i-Soft Clinical Manager summaries and/or written email for complex handovers). All clinical handover must be completed in ISOBAR format.
- When it is identified that a patient requires weekend review (patients on stroke pathway, patients requiring post-fall huddle or review), ensure all relevant handovers are completed to the rostered weekend OT.

Caseload management

- At commencement of shift, print out the i-Soft clinical manager list of the ward's patients and handover notes or Rehab Ward nursing handover.
- Review the patient list and prioritize caseload based on clinical need – please see table 1 for reference.
- Attend Rehab OT morning huddle (0820hrs-0830hrs) to:
 - Provide any relevant handover to OT representative attending daily RAC Ward Morning MDT Patient Flow meeting (see below for details)
 - Running through caseload numbers for the day with team to extend/obtain support where required
 - Obtain any updates impacting on processes or patient care
- An OT representative will attend the daily RAC Ward Morning MDT Patient Flow meeting (0830hrs-0845hrs) to
 - Obtain any updates on changes in patient health (e.g. falls, fevers, unwell),
 - Identify and confirm discharges for the day and anticipated discharges for the following business day, and

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- Confirm plans for the day (e.g. OT home visits, appointments, investigations).
- Patients identified as requiring 1:1 Allied Health Assistants (AHAs) input will be referred via electronic referral sheet.
- OTs will attend team-specific weekly Multidisciplinary Team Meetings (Monday – Blue Team, Tuesday – Red Team, Wednesday – Green Team) to discuss patient progress and discharge planning with other members of the MDT. OTs must be prepared with feedback in relation to the patients' pre-admission baseline, rehabilitation goals, current functional and cognitive status, pressure care needs, falls risk, and any barriers towards discharge home.
- Monitoring of any new admissions to allocated teams throughout the day with the rostered Nursing Coordinator.

Table 1: Prioritisation guidelines for OT caseload on Rehabilitation Ward

Priority Level	Definition of Priority Level
Priority 1	● Medically stable and flagged for same day discharge - requiring OT input ● Inpatient falls (huddle and review required) ● Stroke pathway with clinical risk or between 24-48 hrs and not seen ● New admissions to ward requiring room setup +/- initial assessments and goal setting ● Home Assessments ● Pre-organized meetings (e.g. family meetings, carer training) ● Transfer summaries for patients flagged for same day discharge to RACF or interhospital transfers ● Patients at significant risk of deterioration or not seen in 3 days ● New pressure injuries requiring assessment within 24 hours ● Palliative patients requiring pressure review (air mattress as appropriate)
Priority 2	Medically stable for therapy ● Estimated date of discharge not yet known, ● Still progressing re: mobility (ie bed-bound/full hoist/2xA for transfers), ● Reviewed by OT 1-2 days prior and adequate for AHA input in interim ● Allied Health Reports for patients on TCP-Restorative pathway ● Follow up on home modifications request
Priority 3	● Medically unable to participate in therapy ● Awaiting transfer to residential care/TCP restorative/Kalamunda hospital with nil significant changes in risk.

4. Admission process

4.1 Referral Protocol

All patients admitted to the ward will undergo assessment by the OT team, with the exception of medical outliers. Medical outliers will be triaged and prioritized for assessment based on the OT Medical Ward Policy. Additionally, patients may be re-referred to OT for further review by other disciplines as needed.

4.2 Pre-assessment preparation

1. Review Medical Records:

- Check for diagnoses, comorbidities, surgical history, and precautions (e.g. weight-bearing restrictions).
- Review the patient's observation chart and any relevant scans or pathology results to ensure they are medically appropriate for OT review. Consult medical team if clearance is required.

2. Gather Relevant Background Information:

- Review previous therapy notes if available on Digital Medical Record (DMR), any ISOBAR handovers, and iSoft clinical handover.

3. Prepare Necessary Tools (e.g. Functional assessment tools, ADL equipment, and cognitive screening tests)

4.3 Initial review

- Initial reviews will be commenced on day of admission. New patients who admit to the ward after 1400 hours are to be reviewed by the next business day.
- Complete OT Initial Assessment (using AKMR 61.1 for reference) for new patients. This is not required if a patient has had one completed and information is available on DMR and circumstances are unchanged in the past 12 months. In these circumstances the previous assessment should be noted and referenced in medical record note entry.
- When patients cannot provide an accurate social history due to cognitive decline or medical illness, the patient's next of kin may be contacted for collateral history (with patient's consent).
- If a patient has difficulty conversing in English and has no social support to provide assistance for interpretation OR if the patient requires a cognitive assessment to be completed, interpreter services can be arranged.

4.4 Functional review

The OT will evaluate the patient's physical function (e.g. functional mobility and care needs) and cognitive function through both direct observation and standardized assessments. Activities of daily living (ADLs) are assessed to identify any challenges the patient may face. The OT will also identify potential barriers to function, such as pain, cognitive impairments, or environmental factors. Based on the findings, the OT will develop targeted interventions and a personalized rehabilitation plan, ensuring that therapeutic strategies align with the patient's specific functional needs and goals.

4.5 Room setup (in relation to the patient's function)

- Equipment Provision:
 - Review requirements for seating, showering, toileting, transfers, reducing falls risk and safe positioning of the client – in relation to the patient's function. Where specialized equipment is prescribed, ensure handover is given to nursing staff, recorded on iSOFT and positioning diagrams (where relevant) are visible above the patients' bed.
- Seating Assessment:

- Assessment of the patient's sitting tolerance, positioning, and pressure requirements to ensure the patient has an appropriate seating option and pressure relieving cushion if indicated.
- Pressure Assessments:
 - Completing a skin assessment in liaison with nursing staff to assess for any pressure injuries, reviewing AKG Skin and Pressure Injury Assessment Tool AKMR121.9, and assessing suitability for appropriate pressure relieving devices. For clients at risk of having a pressure injury refer to intervention requirements in the [OT Pressure Injury Prevention and Management Procedure](#)

4.6 Falls risk assessment and management

Upon admission, the patient's functional needs and cognitive status are evaluated to assess their falls risk. The Comprehensive Care Plan (AKMR 1119) should be reviewed and updated, with the OT providing a summary of recommendations for falls prevention strategies. Falls risk assessment and prevention must align with the [Falls Prevention and management guideline](#). For patients identified as having a high falls risk, their suitability for the Safe Recovery Program will be assessed in liaison with the treating physiotherapist. Eligible patients can be referred via the patient board located in the Red Office.

4.6.1 Inpatient falls

- A [Post-fall Huddle](#) will be held following any fall on the ward. Relevant stakeholders, including the medical team, OT, PT, nursing staff, and pharmacist, will review the fall and the interventions implemented to reduce further risk. These interventions must be documented in both the Comprehensive Care Plan and patient records (e.g., low bed, invisa-beam).
- A [Post-fall Review](#) must occur within 24 hours or the next business day, in collaboration with the patient's treating physiotherapist. This review will assess any changes in the patient's functional status or cognition, allowing the team to update the treatment plan, adjust recommendations, and introduce additional falls prevention strategies as needed.

4.7 Cognitive screening

- Undertake best practices for identifying, assessing and managing cognitive impairments such as dementia and delirium in line with AKG [Delirium Clinical Management Guideline](#) and [Cognitive Impairment Screening Procedure](#).
- Review abbreviated mental test score (AMTS) completed by nursing staff and assess if there has been any acute change in cognition and screen for delirium using 4AT delirium tool, and the patient's suitability for a 'TOP 5' – this should be completed with patient's family or loved one.
- Identify need for formal cognitive assessment, evaluate functional impacts and implement person-centered strategies to enhance patient safety.

All of the above assessments and interventions will be regularly monitored and adjusted as the patient's care needs evolve. Ongoing evaluations will ensure that the strategies remain appropriate and effective, with updates made to the care plan as necessary to address any changes in the patient's condition or functional requirements.

4.8 Goal setting and orientation to ward

- Educate patient on rehab process and encourage patient engagement on ward.

- Identify if patients are suitable for the following input: AHA 1:1 therapy, group therapy, Forget Me Not (FMN) Volunteers (Note: All staff can refer to the 'Forget me Not' volunteer program the referral forms are at the ward Nurse coordinator desks. The Forget Me Not program is staffed by trained volunteers who offer comfort and practical assistance to patients most in need, often those living with dementia)
- Discuss patient-centered goals with patient - goal setting with patients will occur within 48 hours of admission. Goal setting will be in conjunction with other allied health team members. Where patients are cognitively impaired family members are invited to participate in goal setting.
- Update patient's communication board with any relevant details or booked assessments

5. Interventions planning and implementation

Formulating plan:

- Cross-refer to other disciplines if this need is identified during OT assessments
- Identify clinical need for further comprehensive OT assessment. These may include:
 - Self-care assessments (e.g. Modified Barthel Index or Functional Independence Measure)
 - Functional assessments (e.g. meal preparation, medication management)
 - Upper limb assessments
 - Cognitive assessments
 - Perceptual assessments
 - Equipment assessments
 - Instrumental Activities of Daily Living
- Home Visiting: When it is identified that a home assessment is required to facilitate discharge planning, the assessment is to be booked as soon as possible (pending clinical status of the patient and functional progress on ward) to allow adequate time for equipment provision and home modifications (if deemed 'essential') to be completed before discharge. (See 4.1 for further details).
- Where further assessment is scheduled, planned assessment times are to be recorded on the patient's communication board and nursing handover (if impacting on nursing care e.g. shower assessments, carer training).
- Assessments that do not have a formal AKMR number will need to be clearly summarized in the progress notes and filed in the correspondence section of the patient's medical file.

5.1 OT-Specific Interventions: (including but not limited to)

- Prescription of pressure care and seating devices,
- Implementation of falls prevention strategies,
- Education on ADL retraining strategies or activity modification strategies,
- Prescription of small aids and equipment,
- Hand and upper limb therapy, including splinting (in consultation with AHS Hand Therapist where required),
- Lymphoedema management (in consultation with lymphoedema-trained OTs at AHS),
- Functional or cognitive rehabilitation programs via allied health assistant.

- Referrals to other disciplines

Assessments and interventions are provided in line with relevant AKG wide procedures. Including [prevention and management of pressure injuries; inpatient falls prevention and management](#) provision of [OT home assessments; provision of aids/equipment and home modifications](#); pressure injury prevention and management and falls prevention and management.

5.2 Allied Health Assistants

Allied Health Assistants work under direct supervision of the OTs and assist with OT intervention to facilitate progress with therapy and discharge planning. AHA input must be relevant to the patient's therapy goals and aim to contribute to the patient's overall progression or maintenance of functional/cognitive status.

Referrals to AHAs are made electronically by 0900hrs daily and should include information including the task to be completed, priority of the task requested, and any relevant information or risks. The OT will provide supervision or training where required. See table below for AHA duties.

Table 2: AHA Duties and corresponding OT responsibilities

Clinical	Including but not limited to:	OT responsibilities & handover to be provided to AHA:
ADL Retraining	Self-Care Retraining Functional transfer practice (e.g. hob) Customized functional exercise programs Meal preparation practice	• Any resources required – if patient has been prescribed with equipment or adaptive aids. • Any precautions (e.g. falls, weightbearing) • Any risks (e.g. pain, falls, mood/engagement) • Height of furniture/hob at home to be simulated on ward • AHA shower practice - if waterproof bags are required to cover wounds/splints
Upper Limb exercises	ROM exercises Resistant based exercise for strength Fine motor activities Coordination activities	• Any resources required e.g. theraputty, theraband, weights, etc. • A handout should be provided to the patient and AHA for their reference e.g. indicating reps/sets • Any precautions (weightbearing/range of motion)
Cognitive Retraining	TOP 5 Daily reorientation Cognitive retraining activities	• TOP 5 – who to contact • Orientation chart – should this be updated only or does patient need to be engaged during reorientation • Handout of cognitive exercises (if applicable)
Equipment	Setting out of equipment for patients discharging	Provide exact details of equipment pt requires: • Equipment • Size (standard, wide, bariatric) • Popliteal height

Clinical	Including but not limited to:	OT responsibilities & handover to be provided to AHA:
Other:	Assisting OT with joint reviews (including 2-person home visits for risk management)	- Indicate time of joint review and likely duration - Brief AHA on purpose of session and task demands - Advise of any specific therapy resources required for the session (e.g. transfer aids, therabands, etc.)
Non-Clinical	Including but not limited to:	OT responsibilities
Other:	Cushions: Fortnightly audit of ROHO cushions and restocking of foam cushion cabinet.	Ensure tracking sheet is kept up to date – ROHO cushions are to be signed out with patient's name, bed number, and prescribing therapist's name.

5.3 Group therapy

Involves structured sessions where patients work together on shared therapeutic goals under the guidance of an OT and/or Allied Health Assistant. These sessions may target physical, cognitive, or psychosocial skills and encourages social interaction. Group therapy complements individualized therapy programs by promoting co-patient support and creating opportunities to practice skills in a collaborative environment.

Table 3: Rehab Ward OT Group Therapy Programs

Group Program	Criteria
Cognitive Group Day: Monday Facilitator: 2x AHAs Group Capacity: 10 pax	- AMTS 5 and above - Mild-mod cognitive impairment - Not on contact precautions - Appropriate to engage in a group setting - Vision and hearing intact or have relevant sensory aids insitu - Sitting tolerance of 1 hour minimum - Patients with clear cognitive rehabilitation goals are prioritized – if group has not met full capacity then other patients can also be considered.
Gross Motor Group Day: Wednesday Facilitator: 2x AHAs Group Capacity: 10 pax	- AMTS 5 and above - Able to follow simple 1-2 stage commands - Not on contact precautions - Appropriate to engage in a group setting - Vision and hearing intact or have relevant sensory aids insitu - Sitting tolerance of 1 hour minimum - No medical contraindications (eg orthopaedic precautions for fractures, severe cardiovascular conditions) impacting on ability to participate in gross motor activities. - Patients with clear functional rehabilitation goals are prioritized – if group has not met full capacity then other patients can also be considered.
Breakfast Group	Sitting tolerance of 30 minutes minimum

Group Program	Criteria
Day: Tuesday & Thursday Facilitator: 1x OT, 1x AHA Group Capacity: 4 pax (Allocated 30-minute sessions)	- No medical contraindications (eg orthopaedic precautions for fractures, severe cardiovascular conditions) impacting on ability to participate - Have clear functional rehabilitation goal to return to managing simple meal preparation
Sensory & Reminiscing Group Day: Friday Facilitator: 2x AHAs Group Capacity: 8 pax	- Mild-mod cognitive impairment or dementia who can recall or discuss past experiences - Patients with severe cognitive impairments who may benefit from sensory engagement and can tolerate moderate environmental changes - Not on contact precautions - Appropriate to engage in a group setting - Vision and hearing intact or have relevant sensory aids insitu - Sitting tolerance of 1 hour minimum

5.4 Home Assessments

OT home assessments may be completed to ensure patients can safely transition to their home environment by addressing environmental barriers and assessing functional and/or cognitive abilities. **Access home visits** assess the home environment for safety and functionality, **functional home visits** evaluate the patient's ability to manage safely at home, and **discharge home visits** confirm readiness for discharge.

Summary of Key Differences			
Aspect	Access Home Visit	Functional Home Visit	Discharge Home Visit
Patient Presence	Patient not present . Family representative present on behalf of patient.	Patient present , brought home for assessment and then returned to the hospital.	Patient present , assessed for risks, remains home if deemed safe otherwise returns to hospital with OT.
Purpose	Identify physical barriers or hazards in the home environment.	Assess how the patient performs ADLs in their home to evaluate safety and functionality.	Confirm readiness for discharge by verifying safety and functionality at home.
Focus	Environment-focused and identify potential home modifications.	Patient-focused: functional and/or cognitive abilities in home environment, task safety, and equipment use.	Transition-focused: ensuring the patient can remain safely at home.
Outcome	Recommendations for home modifications, equipment, and	Adjustments to therapy goals or recommendations based on patient's ability to manage at home.	Patient is either safely discharged home or returns to hospital (bed

Summary of Key Differences			
Aspect	Access Home Visit	Functional Home Visit	Discharge Home Visit
	environmental adjustments.		held open) for revised discharge planning.
Timing	Early or mid-rehabilitation, before the patient's discharge preparation.	Mid-rehabilitation, often before discharge planning, to finalize therapy needs.	Final step for 'high-risk' patients, requiring detailed assessment of risks and supports.
Appropriate Patients	Patients unable to leave the hospital, or whose home requires initial evaluation.	Patients who need assessment in a familiar environment to determine safety and functionality.	Patients with complex functional or cognitive needs, high fall risk, unresolved hazards, or limited support systems.

- When it is identified that a home assessment is required, the assessment is to be booked as soon as possible to allow adequate time for equipment provision installations / modifications to be completed before discharge.
- Where a patient functional home assessment is indicated the occupational therapist will consult with the MDT to ensure patient is appropriate to attend a home assessment during their inpatient stay.
- Home assessments are conducted following the [Occupational Therapy Home Assessments Procedure](#). This includes outlines for conducting home assessments with current inpatients.
- Interventions indicated from home assessment will follow procedures in the [Allied Health Provision of Aids/Equipment and Home Modifications Procedure](#) and the [Occupational Therapy Home Installations, Fabrications and Modifications Procedure](#).
- Home modifications may be deemed necessary and essential prior to discharge when a patient's mobility, safety, or functional independence is compromised, and modifications are required to ensure safe navigation and performance of daily tasks in their home environment and reduce the likelihood of readmission to hospital. If home modifications are identified as necessary for discharge, the OT will promptly notify the patient's treating MDT to ensure any impact on the planned discharge date is addressed. The OT will also inform the treating team of the modification installation timeline once it has been confirmed by the relevant contractors.

5.4.1 What happens if a patient fails a discharge home visit assessment?

A patient may fail a discharge home visit and need to return to hospital if significant safety concerns are identified that cannot be adequately addressed at home. During discharge home visits, the hospital bed is held open for the patient until the OT confirms the patient's readiness for discharge, ensuring continuity of care and preventing premature discharge. This allows the patient to return with the OT to the ward if further assessments are required. Findings are shared with the patient, next-of-kin, and the MDT to ensure all stakeholders are updated. A revised discharge plan is then developed with the multidisciplinary team.

6. Discharge planning

- The occupational therapist will provide relevant feedback on patients' progress towards active goals, risk factors and functional ability. The team will discuss what is needed before discharge and will identify the need for carer trial on ward.

- The OT will organize if family members require extra training on equipment provisions before discharge.
- Where equipment is essential for discharge this will be supplied prior or on day of discharge.
- Where home modifications are deemed essential for discharge the Occupational Therapist will communicate this to the MDT and update the MDT should this affect planned discharge dates.
- All identified referrals for ongoing management will be completed prior to discharge. Where required transfer summaries will be completed to facilities and services including the Transitional Care Program, Rehabilitation in the Home, inter hospital transfers and transfers to Aged Care Facilities.
- Referral for outpatient follow-up will be completed using the relevant outpatient referral form e.g. Community Rehabilitation.

7. Supporting information

The following documents support the implementation of this procedure:

- [Occupational Therapy General medical, Intensive care unit and General Surgery Clinical Interventions Procedure](#)
- [Occupational Therapy Pressure Injury Prevention and Management Procedure](#)
- [Occupational Therapy Home Assessments Procedure](#)
- [Occupational Therapy Home Installations, Fabrications and Modifications Guideline](#)
- [Allied Health Provision of Assistive Technology and Home Modifications Procedure](#)
- [Fall and Fall Injury Prevention and Management Guideline](#)
- [Occupational Therapy: Issue of Bed Rails for Home Use](#)
- [Interpreter and Translator Guideline](#)
- [Delirium Clinical Management Guideline](#)
- [Cognitive Impairment Screening Procedure](#)
- [Occupational Therapy Australia Position Paper: The Role of Allied Health Assistants in Supporting Occupational Therapy Practice](#)

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Comprehensive Care Standard
 - Preventing and Managing Pressure Injuries
 - Preventing Falls and Harm from Falls
 - Preventing Delirium and Managing Cognitive Impairment
- Standard 6 – Communicating for Safety Standard
 - Partnering with Consumers
 - Document of Information

9. Document contact

Enquiries relating to this procedure may be directed to:

Title: Senior Occupation Therapist
 Department: Rehabilitation Ward

Email: Elizabeth.soon@health.wa.gov.au

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3.0	07/03/2025	07/03/2028	Whole document: Included sections for: • Caseload prioritisation guidelines • Run through of OT processes specific to rehab admissions • Identifying different home assessment types and associated criteria • Allied Health assistants roles and referrals from OT (including information for individual and group therapy)

11. Authorisation

This procedure has been authorised and issued by Director Allied Health and Ambulatory Care:

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	07/03/2025
Published date	07/03/2025